

Assessment Criteria

SOAP Note Case Study · scored 1–4 per criterion, weighted to 100 points

Criterion	4 — Exemplary	3 — Proficient	2 — Developing	1 — Beginning	Wt	Score
1. Subjective data Chief complaint, history, patient's own words	Chief complaint, onset, duration, frequency and context all captured. Patient's words quoted where it matters. Medication gap and sleep pattern both recorded.	Chief complaint and main history present and attributed to the patient. One detail thin or paraphrased where a quote was better.	Chief complaint present but history incomplete. Patient statements rewritten as the writer's own conclusions.	No usable chief complaint, or subjective and objective data mixed together.	15	
2. Objective data Vitals, measurements, point-of-care results	All values recorded accurately with units, site, position and cuff size. Repeat blood pressure documented with the interval. Allergy status recorded.	All values present and accurate. One qualifier missing (site, position or timing).	Values present but with errors, missing units, or the repeat reading omitted.	Values wrong, invented, or largely absent.	20	
3. Note structure SOAP organization and chronology	Four sections clearly separated, each holding only what belongs in it. Chronological, dated, timed and signed with credential.	Four sections present and mostly correct. One item filed in the wrong section.	Sections blurred or out of order. Date, time or signature missing.	No recognizable SOAP structure. Reads as an essay.	15	
4. Plan & patient education Follow-up, instructions, escalation	Education is specific, actionable and at the patient's level. Correctly identifies what must be escalated to the provider before the patient leaves.	Education documented and appropriate. Escalation identified but not clearly prioritized.	Education generic ("advised to eat healthy"). Escalation vague or missing.	No education documented, or advice given that is outside scope.	15	
5. Scope of practice Safety-critical	Stays fully within the MA role. Correctly identifies and rewrites the out-of-scope statement. Names a relevant HIPAA consideration for this encounter.	Stays within scope. Identifies the out-of-scope statement; rewrite is slightly off or HIPAA point is generic.	One borderline interpretive statement in the note, or the reflection does not identify the correct issue.	Diagnoses, interprets results, or advises a medication change. Automatic resubmission.	15	

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6. Terminology & abbreviations	Correct terminology throughout. Approved abbreviations only. Nothing from the do-not-use list.	Correct throughout with one minor terminology slip that does not change meaning.	Several terminology errors, or an unapproved abbreviation used.	Terminology errors that change clinical meaning, or do-not-use abbreviations present.	10	
7. Professional writing Clarity, grammar, format, sources	Concise and unambiguous. Clean grammar and spelling. Format and length met. Sources cited.	Clear and readable. A few surface errors. Format met.	Errors slow the reader or create ambiguity. Format or length not met.	Hard to follow. Errors would make the record unreliable.	10	